

STEADY

Return-to-Life Goals

Implementation-Ready Engineering Handoff

Clinical Intelligence Expansion | Document 01 | Repository-grounded against tprawlins-lang/Emdr | branch
claude/gifted-keller-501y5d | September 2026

Build directive

Add individualized functional outcome goals that answer the most important question beyond symptom reduction: what does this person want to be able to do again, and is their real life expanding? Goals are observable, source-backed, longitudinal, and integrated into check-ins, Companion, clinician workflow, Session Prep, and later intelligence engines.

Shared product rule

The clinician decides what clinical meaning and action to accept. Steady may measure, organize, compare, and suggest, but it does not silently turn an association, model output, or behavioral signal into clinical fact.

Repository foundation this handoff assumes

Foundation	Current Steady anchor	Rule for this build
Longitudinal history	src/lib/spine.ts + ADR 0010	Durable state must be event-backed and replayable. Do not create a parallel mutable history.
Tenant/person boundary	src/lib/repository.ts + ADR 0011	Every new durable patient record carries tenant_id and person_id; new paths use TenantContext even while legacy paths are migrating.
Clinician patient workspace	src/app/clinician/member/[id]/*	New clinician surfaces extend PersonShell and existing evidence/review conventions.
Evidence-linked summary	src/lib/clinical/summary.ts	A generated claim without resolvable evidence is withheld. Summary is a reading aid, not the record.
Trajectory	src/lib/clinical/trajectory.ts	Separate scales stay separate; provenance remains visible; do not invent a composite clinical meaning from arbitrary axis alignment.
Work queue	src/lib/clinical/work-queue.ts	Clinician home opens on actionable work, stable server ordering, clear reason, one primary action.
Companion memory	src/lib/companion.ts	Patient/Companion data retains source class and memory exposure rules. AI output is not clinician evidence.
AI Gateway	ADR 0012	New model tasks must route through the gateway with schema validation, task version, provenance, evaluation, and PHI policy.

This series also assumes the Clinician Thoughts / Clinical Memory / Session Prep Engineering Specification v2.1 is implemented or being implemented as the common clinician-memory and evidence layer.

Cross-feature invariants

- **Safety authority stays deterministic.** These features may add clinician decision support but cannot clear, weaken, bypass, or replace the safety engine.
- **Stabilization scope stays intact.** No feature autonomously initiates trauma reprocessing, changes a gated treatment decision, diagnoses, or issues a treatment order.
- **Source classes never collapse.** Patient report, clinician observation, system measurement, formal note, Companion interaction, and model inference remain distinguishable.
- **Associations are associations.** A temporal or statistical relationship is not described as causation unless a clinician explicitly records that judgement from appropriate evidence.
- **No future-data leakage.** Historical reconstruction and point-in-time evaluations use only information Steady knew at the relevant cutoff.
- **Missing data stays visible.** Missing observations do not become zero, normal, compliant, or recovered.
- **Corrections append.** An authorized correction supersedes prior derived state without erasing history.
- **Model output is disposable; evidence and provenance are permanent.** Every material intelligence output points to evidence IDs and task/policy versions.

1. Product outcome

- Replace vague outcome language such as "feel less anxious" with concrete life participation goals when the patient and clinician choose to do so: drive on the freeway, sleep alone, attend a family dinner, return to work, enter a grocery store, resume exercise, tolerate a specific conversation, or reconnect socially.
- Use a five-level individualized goal ladder inspired by functional rehabilitation goal attainment, but do not claim clinical validation unless Steady formally validates the implementation.
- Progress can be reported by the patient, observed by a clinician, or supported by system evidence. These sources remain separate.

- The goal itself is patient-owned and clinician-visible. AI can help draft measurable wording but cannot choose what matters to the patient.

2. Core workflow

```

PATIENT / CLINICIAN IDENTIFIES LIFE GOAL
|
+--> free natural statement
+--> optional AI draft into observable ladder
|
v
REVIEW GOAL + FIVE LEVELS
-2 current/baseline
-1 meaningful partial progress
0 target meaningful function
+1 beyond target
+2 substantially beyond target
|
v
CONFIRM BY PATIENT / CLINICIAN POLICY
|
v
GOAL BECOMES ACTIVE
|
+--> lightweight patient check-ins
+--> Companion may ask contextually
+--> clinician can record observation
+--> evidence matcher proposes relevant source items
|
v
PROGRESS TIMELINE + SESSION PREP + TRAJECTORY

```

3. Goal design rules

- **Observable over abstract.** A level should describe something a person can report or a clinician can recognize.
- **One dimension per goal.** Do not combine "sleep better, work full time, and stop panic" into one ladder.
- **Patient meaning is stored.** Capture why the goal matters in the patient's words. It should not be used as an AI outcome score.
- **Baseline is descriptive, not a judgement.** Level -2 is where the person is when the ladder is set, not failure.
- **Goal level changes are evidence events.** Do not overwrite the current level without preserving the observation that caused the change.
- **Achievement is not cure.** A completed goal says the function changed; it does not prove symptom remission or treatment causation.

4. Database schema

```

CREATE TABLE return_to_life_goals (
  id TEXT PRIMARY KEY,
  tenant_id TEXT NOT NULL,
  person_id TEXT NOT NULL,
  title TEXT NOT NULL,
  patient_statement TEXT NOT NULL,
  why_it_matters TEXT,
  domain TEXT NOT NULL,
  status TEXT NOT NULL CHECK (status IN ('draft','active','paused','completed','archived')),
  created_by_person_id TEXT NOT NULL,
  confirmed_by_person_id TEXT,
  confirmed_at TEXT,
  target_review_date TEXT,
  current_level INTEGER CHECK (current_level BETWEEN -2 AND 2),
  created_at TEXT NOT NULL,
  updated_at TEXT NOT NULL
);

CREATE TABLE return_to_life_goal_levels (
  id TEXT PRIMARY KEY,

```

```

tenant_id TEXT NOT NULL,
person_id TEXT NOT NULL,
goal_id TEXT NOT NULL,
level INTEGER NOT NULL CHECK (level BETWEEN -2 AND 2),
description TEXT NOT NULL,
created_at TEXT NOT NULL,
UNIQUE(goal_id, level)
);

CREATE TABLE return_to_life_observations (
  id TEXT PRIMARY KEY,
  tenant_id TEXT NOT NULL,
  person_id TEXT NOT NULL,
  goal_id TEXT NOT NULL,
  observed_level INTEGER CHECK (observed_level BETWEEN -2 AND 2),
  evidence_class TEXT NOT NULL CHECK (evidence_class IN (
    'patient_reported', 'clinician_observed', 'system_measured', 'model_candidate'
  )),
  source_type TEXT NOT NULL,
  source_id TEXT NOT NULL,
  occurred_at TEXT NOT NULL,
  note TEXT,
  status TEXT NOT NULL CHECK (status IN ('proposed', 'accepted', 'rejected')),
  decided_by TEXT,
  decided_at TEXT,
  created_at TEXT NOT NULL
);

CREATE INDEX idx_rtl_goal_person_status
  ON return_to_life_goals(tenant_id, person_id, status);
CREATE INDEX idx_rtl_obs_goal_time
  ON return_to_life_observations(tenant_id, goal_id, occurred_at DESC);

```

5. Domain taxonomy

Domain	Examples	Notes
daily_living	shopping, cooking, showering, leaving home	Functional participation, not symptom diagnosis.
sleep	sleep alone, maintain routine, return to own bedroom	Goal is behavioral/function; sleep quality measure remains separate.
work_school	return to shift, attend class, complete presentation	Store workload separately if useful.
relationships	family meal, difficult conversation, social outing	Do not infer relationship quality from attendance alone.
mobility_travel	drive, fly, use public transit, visit location	May pair with distress but function is the primary goal.
self_care	medical visit, exercise, routines	Avoid moralized adherence language.
community_recreation	restaurant, church, sport, hobby, event	Patient decides importance.
other	patient-defined	Never force a goal into an incorrect category.

6. Events

Event	Purpose
return_goal.created.v1	Draft goal exists.
return_goal.ladder_set.v1	Five observable levels recorded.
return_goal.confirmed.v1	Goal becomes active.
return_goal.observation_recorded.v1	Source-backed evidence about current function.
return_goal.level_changed.v1	Accepted observation changes current projection.
return_goal.revised.v1	Goal wording/ladder revised; prior version remains reconstructable.
return_goal.completed.v1	Clinician/patient marks meaningful target complete according to policy.
return_goal.archived.v1	Goal no longer active without erasing history.

7. AI Gateway tasks

Task	Purpose	Hard boundary
return_goal.draft_ladder	Turn natural patient goal into proposed observable -2..+2 levels	Patient/clinician must review; do not choose goal or invent baseline.
return_goal.match_evidence	Find authorized sources that may relate to an active goal	Proposes evidence only; no automatic level change.
return_goal.summarize_progress	Concise source-backed progress wording	Every statement cites accepted observation IDs.

8. Goal draft JSON

```
{
  "title": "Grocery shopping alone",
  "domain": "daily_living",
  "patientStatement": "I want to shop without needing someone with me.",
  "whyItMatters": "I want my independence back.",
  "levels": [
    {"level": -2, "description": "Cannot enter the store alone."},
    {"level": -1, "description": "Enters alone for a few items, leaves early if distress rises."},
    {"level": 0, "description": "Completes a normal grocery trip alone."},
    {"level": 1, "description": "Completes normal trips alone across different stores/times."},
    {"level": 2, "description": "Shops alone comfortably and can stay when plans change."}
  ]
}
```

9. UX surfaces

- **Patient goal setup:** natural statement first, optional AI organization second. Show levels in plain language, not clinical scoring language.
- **Patient Today:** at most one lightweight relevant goal prompt when context supports it. Do not create a daily performance streak.
- **Companion:** may ask about a goal when relevant, but should not nag or turn every conversation into outcome collection.
- **Clinician patient overview:** compact Return-to-Life card: active goals, current accepted level, most recent evidence, change since last review.
- **Goal detail:** five-level ladder, evidence timeline, source labels, revisions, and what changed.
- **Session Prep:** "Life goals" section showing what moved, stalled, or has new evidence since last encounter.

10. Evidence acceptance logic

```
source arrives
|
+-- explicit patient goal check-in --> accepted patient_reported observation
+-- clinician directly records goal observation --> accepted clinician_observed
+-- system event maps exactly to goal criterion --> system_measured candidate/accepted by policy
+-- AI semantic match --> model_candidate ONLY
                        |
                        v
                    human review if needed
                        |
                    accept / reject / ignore
```

11. Integration with existing Steady

- Profile goals_json is legacy/high-level preference data. Do not overload it with longitudinal goal state. New Return-to-Life goals become the durable functional outcome model.
- Program plan may reference active Return-to-Life goals as context, but a wellness program plan cannot redefine them or convert them into treatment targets automatically.
- Clinician Thoughts extraction may create a candidate follow-up or goal observation when the clinician says "she drove herself here today" and an active driving goal exists. It still preserves source and requires the correct approval semantics.

- Response Fingerprint later links interventions to goal observations as one outcome dimension.
- Recovery Trajectory later consumes accepted goal level history as the primary function lane.

12. Security and privacy

- Goal titles and why-it-matters text may be highly sensitive. Do not send them to general telemetry.
- Semantic matching is patient-scoped before vector retrieval.
- Patient may see their own active goals; clinician visibility follows care authorization and consent.
- Model-drafted goal language must not be saved as patient-owned language until confirmed.

13. Implementation phases

Phase 1 - Goal model and manual workflow

Build

- Schema + events + repository
- Patient/clinician manual goal creation
- Five-level ladder
- Goal detail timeline

Definition of done

- Manual end-to-end goal works without AI
- Replay reproduces current level and revisions

Phase 2 - Evidence capture

Build

- Goal check-in prompt
- Clinician observation command
- Session Prep adapter

Definition of done

- Patient and clinician evidence remain source-distinct
- Session Prep shows current goal context

Phase 3 - AI assistance

Build

- Gateway ladder drafting
- Evidence matcher
- Progress summary validation

Definition of done

- No model candidate changes a level automatically
- Every summary statement cites observation IDs

Phase 4 - Downstream contracts

Build

- Expose goal projection API for Response Fingerprint and Trajectory
- Command Center signal adapter for meaningful stall/reversal later

Definition of done

- Stable versioned interface documented and tested

14. Acceptance criteria

- Patient can create and understand a concrete life goal without learning scoring terminology.
- Goal has five observable levels and an auditable current level.
- AI cannot invent a baseline, choose the goal, or silently advance progress.
- Patient report and clinician observation display differently.
- A completed life goal does not create a diagnosis/remission/treatment-effect claim.
- Session Prep and clinician patient overview expose source-backed goal change.
- Cross-tenant and cross-patient attacks fail for goals, observations, semantic matching, and exports.

Appendix A. File-level implementation map

Path	Change
src/lib/clinical/return-to-life.ts	NEW domain/read model.
src/lib/clinical/return-goal-evidence.ts	NEW source adapters and evidence matching.
src/app/clinician/member/[id]/goals/page.tsx	NEW clinician goal view.
src/components/clinical/ReturnToLifeCard.tsx	NEW patient overview card.
src/app/app/goals/* or existing patient plan surface	Patient goal workflow; exact route should follow current app IA.
src/lib/spine.ts	Add Return-to-Life event helpers/schema registrations.
src/lib/projections.ts	Add goal projection/replay.
src/lib/db.ts + scripts/pg-schema.sql	Add/mirror schema + tenant isolation.
src/lib/ai-gateway/*	Register goal tasks.
tests/return-to-life*.test.ts	Domain, security, replay, AI golden cases.